Mr. Nitish Kashyap

Age / Sex : 25 YRS / M

Referred by : SHRI RAM NASHA MUKTI KENDRA

Reg. no. : 6979

6979

Registered on : 20/10/2026 03:49 PM

Collected on : 20/10/2026 10:30 AM

Received on : 20/10/2026 11:00 AM

Reported on : 20/10/2026 03:52 PM

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HAEMATOLOGY

COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**H**	**18.1**	**g/dl**	**13 - 17**
TOTAL LEUKOCYTE COUNT		9,400	cumm	4,800 - 10,800
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS		67	%	40 - 80
LYMPHOCYTE		23	%	20 - 40
EOSINOPHILS	**H**	**08**	**%**	**1 - 6**
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	< 2
PLATELET COUNT	**L**	**1.00**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT		4.91	million/cumm	4.5 - 5.5
HEMATOCRIT VALUE, HCT	**H**	**50.1**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV	**H**	**102.0**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH	**H**	**36.9**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**36.1**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		10.8	fL	6.5 - 12
R.D.W. - SD	**H**	**53.6**	**fL**	**39 - 46**
R.D.W. - CV	**H**	**15.3**	**%**	**11.6 - 14**
P-LCR		33.7		19.7 - 42.4
P.D.W.	**H**	**16.9**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters:

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

BIOCHEMISTRY

LIVER FUNCTION TEST (LFT)

TEST		VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)	H	1.34	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)	H	0.31	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)	H	1.03	mg/dl	0.2 - 1
SGOT (AST)	H	75.14	U/l	0 - 37
SGPT (ALT)	H	80.67	U/l	13 - 40
SGOT/SGPT Method: Calculated		0.93		
SERUM ALKALINE PHOSPHATASE		98.47	U/l	44 - 147
SERUM PROTEIN		7.14	g/dl	6.4 - 8.3
SERUM ALBUMIN	H	5.38	g/dl	3.5 - 5.2
GLOBULIN	L	1.76	g/dl	1.8 - 3.6
A/G RATIO	H	3.06		1.1 - 2.1

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in: Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

KIDNEY FUNCTION TEST (KFT)

TEST	VALUE	UNIT	REFERENCE
RANDOM BLOOD SUGAR	104.5	mg/dl	70 - 140
SERUM UREA	38.57	mg/dl	19 - 45
SERUM CREATININE	1.03	mg/dl	0.72 - 1.43
SERUM PHOSPHORUS	4.45	mg/dl	2.5 - 4.5
SERUM URIC ACID	6.87	mg/dl	3.5 - 7.2
SERUM SODIUM	136.1	mmol/L	136 - 146
SERUM POTASSIUM	4.55	mmol/L	3.5 - 5.1
ICALCIUM	1.15	mmol/l	1.13 - 1.33
PH	7.415		

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level

1. Pre-renal, renal, and post-renal azotemia 2. Large amount of dietary meat 3. Active acromegaly and gigantism	1. Pregnancy 2. Increasing age (reduction in muscle mass)

~~~ End of report ~~~

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